

Difficult Conversation Preparation Guide

Frameworks for Sensitive End of Life Discussions

BEFORE THE CONVERSATION

Difficult conversations in hospice are not about having the right words. They are about having the right posture: listening more than talking, being comfortable with silence, and letting the other person lead when they are ready.

Preparation Checklist

- ☐ Review the patient situation and any clinical details you have
- ☐ Identify who will be in the conversation and their likely concerns
- ☐ Prepare 2 to 3 open ended questions rather than a script
- ☐ Check your own emotional state before entering the room

COMMON SCENARIOS

When a Family Says "We Are Not Ready"

This is the most common thing you will hear. Resist the urge to convince them otherwise.

- Acknowledge: "That makes complete sense. This is a big decision."
- Explore: "Can you tell me more about what feels like it is not the right time?"
- Educate: "Some families find it helpful to at least understand what services are available, with no pressure to decide today."

When a Physician Resists the Hospice Conversation

Physicians often feel that discussing hospice means they have failed. Approach with respect for their clinical judgment.

- Acknowledge: "I understand there is still a lot you are doing for this patient."
- Explore: "Have the family members expressed any concerns about the patient's comfort?"
- Educate: "Hospice can actually work alongside your care plan to support symptom management."

When a Discharge Planner Is Overwhelmed

Discharge planners manage dozens of cases at once. Respect their time and make things simple.

- Lead with: "I know you are busy. I just wanted to check if there is anyone I can help with today."
- Be specific: "I noticed Mrs. Johnson has been here for 12 days. Would it be helpful if I spoke with the family?"

- Follow through: Everything you promise, deliver on time, every time.

LANGUAGE TO USE AND LANGUAGE TO AVOID

Use

- "Tell me more about what is happening."
- "What matters most to you right now?"
- "How can I be most helpful to you today?"
- "There is no pressure to make any decisions right now."

Avoid

- "You need to consider hospice." (Too directive)
- "Most patients at this stage..." (Generalizing their situation)
- "If I were you..." (Making it about you)
- "They qualify for hospice." (Clinical language that feels cold)

THE MOST IMPORTANT THING

In difficult conversations, your presence matters more than your words. Being calm, patient, and genuinely interested in what the other person needs will carry you further than any scripted response.